



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رباعياً : شرين علي محمد جاد

المحافظة : بورسعيد

الحي / المدينة : الشرق

العنوان : الشراييه

رقم البطاقة : 27908030101701 رقم التليفون : 01128896060

تاريخ تسجيل الإستمارة : 26-09-2026

العمر (تاريخ الميلاد) : 03-08-1979 المهنة : ربه منزل

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

قسم العلاج :
الاورام

التعليق على التشخيص : -----

خطاب إداري

هذا الرقم القومي يتبع منظومة التأمين الصحي الشامل جهة الاختصاص



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Name: شرين علي محمد جاد National ID: 27908030101701	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

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جمهورية مصر العربية
بطاقة تحقيق الشخصية



لترين
علي محمد جواد
١٩ ش النور - هاشم اغا
الشرابية - القاهرة

٢٧٩٠٨٣٠١٠١٧٠١

HX5516489

٢٠١٩/٠٢ ٢٧٩٠٨٣٠١٠١٧٠١

بدون عمل

أنشى
مسجلة
اسمها حسين السيد بدر
البطاقة سارية حتى ٢٠٢٩/٠٢/٢٨





Name: شرين علي محمد جاد			
MR. : 1111			
Detailed Medical Report			
		السيد	
تجبة طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 47-year-old individual who was diagnosed with right-sided oligometastatic breast cancer in October 2022. The initial presentation included a small bony lesion at D8 on CT-CAP. The core biopsy performed in 2022 demonstrated invasive ductal carcinoma, grade II. Hormone-receptor testing showed estrogen receptor 8/8 and progesterone receptor 5/8, with HER2 positivity and a Ki-67 proliferative index of 40 %. Subsequent imaging identified a non-conclusive hepatic focal lesion measuring 14 × 15 mm in segment VIII on CT (February 2023) and a 9-mm focal lesion in segment V on MRI (April 2023) thought to represent a cyst. Spinal MRI in November 2023 revealed several osseous lesions at L4 and L5, likely metastatic, while a bone scan in March 2024 was negative. Follow-up imaging in 2026 demonstrated progressive skeletal disease with multiple osseous lesions throughout the skull, sternum, scapulae, ribs, vertebrae, sacro-iliac joints and pelvis, and pathological anterior wedge fractures of L1, 2 and 4. The multidisciplinary team initially recommended in December 2022 a regimen of letrozole (Femara), trastuzumab (Herceptin) and goserelin (Zoladex) for three months, given the patient’s refusal of chemotherapy. The patient subsequently completed 27 cycles of intravenous trastuzumab by August 2024, continued letrozole and goserelin through June 2025, and underwent right modified radical mastectomy in January 2025, which revealed residual viable IDC GII with DCIS, ypT1cN2b (4/9) and clear margins. In March 2025 the MDT planned six cycles of TCHP followed by postoperative aromatase-inhibitor therapy and Zoladex; however, only six cycles of docetaxel-carboplatin were administered, with no anti-HER2 therapy, the last cycle in October 2025. The patient was restarted on letrozole and goserelin in November 2025 and continues this regimen. Current complaints include bilateral loin pain, vomiting and occasional hematemesis, correlating with extensive metastatic bone disease documented on recent CT-CAP, bone scan and MRI studies.			
تحريرا في ,			
		التاريخ : 23-09-2026	
الوقت :		الطبيب المعالج :	
يعتمد ,			

Patient Name	: الأستاذ / شربين على محمد جاد	Request No.	: 13001787
Patient No.	: 27908030101701	Request Date	: 06/01/2025
Path No.	: 72/25	Verification Date	: 16/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - ميانره
Age	: 45 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Right MRM measured 24x17x8 cm with covering skin ellipse measured 16x22 cm.
- * Tumor extent: An ill defined greyish white rubbery tumor bed site.
- * Size: 1.5 x0.5 x0.5 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 4 cm away from deep margin.
- * Lymph nodes number and size: Fibrofatty tissue piece measured 12x5x2 cm. Careful repeated dissection entangling only 9 L.Ns up to 2x1.5 cm.

Microscopic Examination:

- * Histological type: Residual viable invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Maximal tumor diameter 1.5 x 0.5 cm
- Actual tumor cellularity: 25%, tumor necrosis index: 75%
- * Ductal carcinoma in situ: Positive DCIS (5%).
- * Skin involvement: Free nipple and areola.
- * Lymph node involvement: Positive in (4/9) with extranodal extension. Maximal tumour diameter 13 mm. Two lymph nodes showed effect of therapy.
- * Resection Margins: Free deep margin.
- * Histologic response to therapy: Partial pathological response.
- * Microcalcifications: No.
- * Additional pathologic findings: Fibrosis, hyalinosis (Effect of therapy).

Verified by :

DR.Menna Nabil

Printing Date : 19/02/2025 12:13:25

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



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Age	: 45 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Right breast cancer post NACT, MRM specimen:

- * Residual viable invasive duct carcinoma G II with focal DCIS (5%).
- * Free nipple and areola.
- * Lymph nodes were positive in (4 / 9) with extranodal extension.
- * Free deep margin.
- * Partial pathological response to therapy.
- Stage: ypT1c, ypN2a.
- RCB: 3.418, RCB class III.

Signature
Dr. Marwa Mahmoud.
Dr. Duaa Mohammed.

Verified by :
DR.Menna Nabil

Printing Date : 19/02/2025 12:13:25

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Sherine Ali Mohamed Gad	Date : 08/10/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 45 Year(s)
Medical ID: 3881/2022	Pat. ID: 18987

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:

- As compared to previous study dated 27/03/2024.

1. Chest:

• **Stationary course regarding the previously seen:**

- Right LIQ area of parenchymal distortion despite being partially out of field.
- Right axillary prominent level I and II lymph nodes, the largest measures 1.8 x 1.4 cm.
- Few tiny scattered mostly subpleural pulmonary nodules ranging between 2-3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is still seen mildly enlarged with slightly inhomogeneous CT texture. No hypodense hepatic focal lesions could be appreciated or contour irregularities.
- Patent homogeneously enhancing portal vein and its major branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions on non-contrast basis.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.

3. The scanned skeleton:

- Stable multi-level vertebral bodies sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- Follow up study showing stationary course since the last study dated 27/3/2024.

Dr. Mostafa Abdelwahab, MSc

Dr. Wafid Mohamed, M.D.

اسم المريض: شيرين علي محمد	PM تاريخ الحالة: 10:20:35 10/09/2026
الطبيب المعالج:	كود أمر الشغل: 105365
كود المريض: 59798	النوع: Female

MULTISLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. IV contrast was used.

FINDINGS:

****Reported patient with breast cancer.**

- The liver is average in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No ascites.
- Scanned lung bases show middle lobar small consolidative opacity with parenchymal bands.
- Diffuse heterogenous density of the lower thoracic and lumbar vertebrae as well as the iliac bones and sacrum showing mixed lytic and sclerotic lesions likely metastatic. Anterior wedging of L1, L2 and L4.

OPINION:

- **Diffuse heterogenous density of the lower thoracic and lumbar vertebrae as well as the iliac bones and sacrum showing mixed lytic and sclerotic lesions likely metastatic.**
- **Anterior wedge fractures of L1, L2 and L4, likely pathological.**

Dr. Asmaa Ali, MD

اسم المريض: شيرين علي محمد	PM تاريخ حاله: 10:20:35 10/09/2026
الطبيب المعالج:	كود أمر الشغل: 105365
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MRI OF THE DORSAL AND LUMBAR SPINE

TECHNIQUE:

- Sagittal STIR, T1 and T2 WIs
- Axial T1 and T2 WIs

FINDINGS:

****Reported patient with breast cancer.**

- Widely scattered marrow infiltrative lesions are seen within almost all the dorsal, lumbar and sacral vertebrae. They appear of low T1 and heterogenous high T2 and STIR signal. No definite extraosseous soft tissue component.
- L1, L2 and L4 anterior wedging.
- Similar lesions are seen within both iliac bones.
- D4-5 central disc protrusion is seen effacing the central CSF space.
- L4-5 posterior disc bulge is seen indenting the theca and partially encroaching upon the neural foramina.
- Normal dimensions of the spinal canal.
- Normal size and signal of the spinal cord.

OPINION:

- **Reported patient with breast cancer shows widely scattered marrow infiltrative lesions as described, likely metastatic.**
- **L1, L2 and L4 anterior wedge compression fractures, likely pathological.**

Dr. Asmaa Ali, MD

09/20/2026

Clinical Data:

1/2

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Dr. Asmaa Ali, MD